

COMMUNICATION SKILLS

INTRODUCTION

- Rapport building is the process of developing an honest relationship build on trust, mutual understanding, and shared empathy with a desire to meet the needs and interest of other members in the relationship.
- Information, education and communication (IEC) is an important approach to bring about changes in the knowledge, attitude and behaviour of the people.
- Education - it consists of educating or motivating the people to change their lifestyle or behaviour for betterment of their health.

COMMUNICATION

1. It is a complex process in which a source of information (sender) gives the information (message) through various channels (media) to the audience (receiver) and in turn gets feedback to know the effect of the process
2. The process of transmitting information and common understanding from one person to another

NEED FOR COMMUNICATION IN HEALTH

1. **Clinical** - a fruitful doctor patient communication is the most important clinical function required in building a therapeutic doctor - patient relationship
2. This is important as this can lead to the delivery of better healthcare
3. Patient dissatisfaction and complaints are due to the breakdown in doctor patient communication

GOALS OF CURRENT DOCTOR PATIENT COMMUNICATION

- Creating a good interpersonal relationship
- Facilitating exchanges of information
- Including patients in informed decision making

PUBLIC HEALTH

Many of the threats to public health are rooted in human behaviour. Public health programs are based on two specific pillars -

1. Supply of services
2. Its utilization and demand by community members

COMMUNICATION PROCESS

- **Sender** - it is the communicator or the originator of the message and should know the audience, the message to be given and the methods or channels to communicate. The effect of the communication depends upon the efficiency of the communicator
- **Message** - It is the 'technical know-how' of the information being communicated to the audience. They should understand, accept and act upon. The message should be clear, understandable, interesting, meaningful, related to the objective, specific, precise and acceptable by the audience.
- **Media** - it is the channel which bridges between the sender and the receiver. There are three systems of media -



- Interpersonal communication -
 - Face to face verbal or non-verbal exchange of information, idea and feelings between two or more people
 - This is very effective
 - Improved doctor - patient communication leads to increase involvement and adherence to recommended therapy

Key elements of interpersonal communication

- Caring : rapport building
- Problem solving
- Counselling

TECHNIQUE THAT HELPS THE PROVIDERS IMPROVE CLINT PATIENT INTERACTION

- Effective questioning
- Active listening
- Reflection or echoing
- Summarizing or paraphrasing
- Praise or encourage
- Giving information

MASS MEDIA

Tv, radio, printed media etc.

Advantages - large number of people can be reached in a short time.

Disadvantages - not very effective in bringing about the changes

TRADITIONAL OR FOLK MEDIA

Folk dance, songs, drama, puppetry, harikatha, burrakatha (in AP), nautanki (in UP) etc. these types are close to cultural values and therefore very effective.

AUDIENCE

- The receiver may be a single person or a group of people.
- If the audience is a homogeneous group and have a common interest, it is called 'controlled audience'.
- Otherwise it is called 'uncontrolled or free' audience (that is a group which has gathered out of curiosity)

FEEDBACK

It is the reverse flow of information or remarks made by the audience about the message to the sender so that the sender can make modifications and improve the communication to make it more effective and acceptable to the audience.

TELECOMMUNICATION AND INTERNET

- This is done by using electromagnetic instrument and communication is done over distance
- Example - radio, tv, internet etc. telephone, telex and telegraph are known as "point to point" telecommunication system
- Launching of sattellites and advanced technology in telecommunication system

STEPS IN IEC (information, education, communication)

PLANNING

- Situational analysis - studying the existing national policies and the organisational structure for the modification or implementation if any.
- Understanding the underlying philosophy. E.g. use of condoms in preventing AIDS
- Identification of target groups such as only men or only woman or couples or youth or parents or teachers etc
- Establishment of objectives, goals, targets (indicator)

IMPLEMENTATION OF IEC ACTIVITIES

Planning ways to ensure that the materials reach the target audience.

- Monitoring - regular checking of the programmed activity. This helps to get the feedback for the improvement.
- Evaluation - this consists of analysing the information periodically about the impact of the program to know the effectiveness

PREPARATORY ACTIVITIES CONSIST OF THE FOLLOWING -

- Developing linkage to the other existing organisation like rotary and lions club, mahila mandals, education department, PWD etc.
- Arranging support activities like advocacy counselling and getting materials and money.
- Conducting effective training to prepare resources
- Knowing the need of the target groups such as posters, matter for TV or radio etc.
- Choosing the appropriate media or channel for cost effectiveness.
- Preparing appropriate IEC material

TYPES OF COMMUNICATION

1. One way communication

It is also called "**didactic method**". In this type, the message flows in only one direction, i.e. from the communicator to the audience, e.g. giving lectures in a classroom. The demerits are-

- There is no participation from the audience
- There is no feedback
- Information is imposed
- Learning is passive
- Less likely to influence the human behaviour

2. Two way communication

It is called socratic method. In this type both the communicator and the audience take part. Audience may ask questions and they may give their own suggestions. Thus the communication becomes active, effective and democratic. This method is more likely to influence the human behaviour

3. Verbal communication

This is giving the information by talk and not by showing written printed matter. It is more persuasive.

4. Non-verbal communication

In this type, the message is communicated not by talk buy by body movements, posture, facial expressions (smile, sorrow, staring etc.) often this is also effective

5. Formal communication
This consists of forms like toys, games, songs, etc.
6. Informal communication
It is by rigid courses
7. Visual communication
Consists of charts, posters, pictures, graphs, tables, maps, etc.

BARRIERS OF COMMUNICATION

There will be certain barriers between the educator and the audience, which will interfere with the communication

Physiological barriers - difficulties in hearing, language, expression.

Psychological barriers - emotional disturbances, neurosis, low level of intelligence, language difficulties.

Environmental barriers - noise, overcrowding, invisibility.

Cultural barriers - illiteracy, customs, beliefs, religion, attitude, poverty, cultural difficulties between rural and urban etc.

Semantic barrier - this occurs due to the difference interpretation of meanings of words in messages

Linguistic - India is a land of many languages and dialects. Difference in knowledge and understanding of these between the sender and receiver of the messages can act as a barrier.

METHODS IN HEALTH EDUCATION

1. Individual and family level
Detail information can be given about illness such as causation, mode of transmission, complications, treatment, prevention, sanitation and hygiene to individual and family level.
2. Group level
Lecture - merits - it is simple, accepted and one-way method
Demerits - audience may get bored and lose the interest
Demonstration - preparation of ORS and accurate method to use condom
3. Group discussion
The group usually comprises of 6-12 persons with common interest and almost of the same education level
They all seat in a circular fashion facing each other. Group "ladies", initiates the subject and manages the discussion by preventing tangential conversation, motivates everyone to participate and finally summarizes the entire discussion.
A recorder prepares a sociogram and a report on the discussion and on the decision taken.

PANEL DISCUSSION

- panel discussion - a group of 4-6 experts in a particular topic get around the table in the presence of audience.
- Symposium - it is also a lecture form but the difference is that different speakers (experts) give lecture on different topics of the same subject.
- Seminar- in this method of health education, one expert speak about the different components of the same subject, to a group, having a common interest or discipline

- Workshop - it consists of group of members, who are subdivided into small groups, each of group 4-6 members.
- Conference - it is a get together of experts or learned people being held usually once in a year wherein the recent advance or the research work discuss and presented in front of the audience.
- Role playing (sociodrama) - education is given by a group in the form of drama, it is more effective and more impressive.
- Brainstorming - a group of 5-20 people participate collecting and recording the ideas or suggestion of the member and finally the decision is made
- Tutorial - a small group of learners and guided by a teacher to help clear doubt and confusions, improve understanding of the subject.
- Colloquy - in this method of group discussion, the members from the audience initiate discussion by asking questions to the experts on the stage, who will answer on the various aspects
- Debate - in this type of discussion method, two sets of speakers talk on "for" and "against" a particular resolution or a statement and give their opinion.

COMMUNITY LEVEL (APPROACH)

1. It consists of educating the whole community (mass approach)
2. Television
3. Radio
4. Press material
5. Folk medias
6. Role playing
7. Health magazine

LEVEL OF IMPACT OF COMMUNICATION

Level of impact have been identified by centre for disease control and prevention

- Individual - the individual is the most fundamental target health related changes, since it is individual behaviours that affect health status.
- The social network - an individual's relationships and the groups to which an individual belongs can have a significant impact on his or her health
- The organisation - organisation like associations, club, civic groups. Organisations can carry health messages to their membership, provide support for individual efforts and make policy changes that enable individual changes.
- The community - community level initiatives are planned and led by organisations and institutions that can influence health- schools, worksites. Healthcare settings community groups and government agencies.
- The society - society as a whole has many influence on individual behaviour, including norms and values, attitudes and opinions, laws and polies and the physical, economic, cultural and information environment.

APPLICATION OF HEALTH COMMUNICATION

- IEC - stands for information, education, and communication
it aims to generate specific awareness to target audience
IEC is usually done for all health programs in three stages - planning, implementation, evaluation.

- BCC - behavioural change communication attempt to generate awareness among targeted recipients with an ultimate aim to change their behaviour
- SBCC - special BCC is an interactive, researched, planned and strategic process with an aim to change social conditions and individual behaviours
- Interpersonal communication - group discussion and street plays
- Mass - TV, radio, newspaper
- New information technologies - social media platforms using mobile phones
- Success story - "communication" the game changer in success of polio.
India achieved one of the biggest achievement in global health. It was certified as polio free by the WHO after 3years without an endemic case of polio.
Help by world bank, UNICEF
The social mobilisation strategy was the game changer in this movement against, polio in india.

HEALTH EDUCATION

The declaration of alma- ata (1978) define health education as " a process aimed at encouraging people to want to be healthy, to do what they can individually and collectively to maintain health and to seek help when needed.

OBJECTIVES OF HEALTH EDUCATION

- To encourage the people to adapt healthy lifestyle
- To encourage them to make best use of available health services. To encourage them to change their attitude towards their own health.
- To improve the health of the family and the community at large

APPROACHES IN HEALTH EDUCATION

1. Regulatory approach - It is legal approach made by the government to alter the behaviour of people for the betterment of their health. E.g. cigarette and other tobacco product ACT (COTPA), 2003.
Merit - this is the simplest and quickest way to improve the health of the people.
2. Service approach - this consists of providing health care services to the individual doors (as in pulse polio immunization), based on the assumption that people would use them to improve their own health.

PRINCIPLE OF HEALTH PROMOTION AND GLOBAL HEALTH INITIATIVES

- Health promotion is fundamentally rooted in the ideas of the fairness, active involvement and empowering individuals.
- It must play a proactive role in advancing world-wide initiatives aimed at achieving health equity and sustainable development, as outlined in the united nation's millennium development goals (MDGs)
- The world health organisation's commission on the social determinants of health highlights the critical social factors influencing health outcomes.

DEFINATION

- Health promotion is the process through which individuals gain greater influence over their health and work towards enhancing it.

- It is a broad social and political endeavor that not only focuses on empowering individuals by building their skills and abilities but also aims to transform social, environmental and economic factors to reduce their negative effects on both public and personal health.
- Ultimately, health promotion enables people to take charge of the various factors that determine their health, leading to overall improvement in well-being.
- Any deliberate effort aimed at fostering learning related to health or illness, resulting in a lasting transformation in a person's skills or attitudes
- Freire emphasizing that empowerment education hinges on critical dialog, which involves engaging in a process of passing problems, rather than simply trying to solve them.
- The authority to define health issues and needs rightfully belongs to those who experience them first hand.
- Individuals marginalized by social structures must take the lead in crafting strategies that enable them to gain greater control and important resources.
- Empowerment is neither solely an internal process. (I alone can empower myself) nor something imposed externally (we must empower the group) ; it is a shared and dynamic process.
- Collaborative power dynamics do not distinguish the specialized knowledge and skills of health professionals

DEVELOP PERSONAL SKILLS

- Supporting growth in personal and social spheres by delivering vital information, health education, and life skill development
- While it is important to remain cautious of the pitfalls and constraints of focusing solely on individual skill-building, we should also reconnect with the most innovative and forward-thinking practices in this field
- Ensuring everyone has equal opportunity to receive health care by eliminating obstacles related to location, social status, financial means, or cultural differences (universality and accessibility)
- Promoting collaborations between health services and other sectors like housing, education and employment to create a holistic approach.

MILESTONES IN GLOBAL HEALTH PROMOTION

- 1974 - launch of the calonde report, a foundational health policy document.
- 1986 - the inaugural international conference on health promotion convenes in Ottawa
- 1988 - the second conference takes place in Adelaide.
- 1991 - the third gathering is held in Sundswall.
- 1997 - the fourth conference meets in Jakarta.
- 2000 - the fifth international conference is hosted by Mexico.
- 2005 - Bangkok welcomes the sixth conference.
- 2009 - Nairobi hosts the seventh global health promotion conference.
- Upcoming - details on the eighth conference's location, date, and purpose are yet to be announced.

PRIMARY HEALTH CARE APPROACH

This is radically a new approach, wherein the health care service is basic, essential provided by the non-medical persons, starting from the people, the by people, of the people and for the people.

Based on principles like equitable distribution, community participation, inter sectoral approach and appropriate technology, thus making the service highly successful.

DEMERITS

- The disease can be controlled but not eradicated by legislation.
- The legislation cannot force the people to act.
- This is how disastrous sterilization campaign during national emergency in 1976, led to congress failure in 1977 election. Regulatory approach will not be successful unless people are ready to accept the law

MODEL OF HEALTH EDUCATION

1. **Medical model** - medical model primarily interested in the recognition and treatment of disease (caring) and technology advances to facilitate the process.
 - a. It is concerned with disease (as defined by the doctor) or opposed to illness (as defined by the clients)
 - b. Health education developed along the lines of the bio-medical virus of health and diseases
 - c. The people would act on the information supplied by the health professionals to improve their health.
 - d. Demerits - in this model, social cultural and psychological factors were thought to be of little or importance. The medical model did not bridge the gap between knowledge and behaviour.
2. **Motivational model** - when people did not act upon the information they received, health education started emphasizing "maturation" as the main force to translate health information into desired health education.
 - a. But the adaption of new behaviour or idea is not a simple act.
 - b. For adaption passed three stages -
 - i. Awareness - internal
 - ii. Motivation - education, decision making
 - iii. Action - adoption or acceptance
3. **Social intervention model** - social environment which shapes the behaviour of individual and the community.
 - a. It is often found that people will not readily accept and try something new or novel until it has been legitimated by the group to which they belong.
 - b. The importance of group support in helping reaching the decisions and taking action.
 - c. An effective health education model is based on precise knowledge of human ecology and understanding of interaction between the cultural, biological, physical and social environment factors

CONTENT OF HEALTH EDUCATION

1. **Human biology** - understanding health, demands an understanding of the human biology i.e. the structure and functions of the body, how to keep physically fit - the

benefit of exercise, rest and sleep.

- a. The effect of alcohol, smoking and drugs on the body
2. **Nutrition** - the aim of nutrition education is to guide people to choose optimum and balanced diets, remove prejudices and promote good dietary habits
 - a. Nutrition education is a major intervention for the prevention of malnutrition, promotion of health and improving the quality of life.
3. **Hygiene** - this has two aspect - personal and environmental.
 - a. The aim of personal hygiene is to promote standards of personal cleanliness within the setting of the condition where people live. Training in personal hygiene should begin at early stage and must be carried through school age.
 - b. Environmental hygiene - domestic and community
4. **Family health** - the family is the first defense as well as the chief reliance for the well being of its members. The role of family in health promotion and in prevention of disease, early diagnosis and care of the sick is of crucial importance.
5. **Disease prevention and control** - drugs will not alone solve health problems without health education, a person may feel sick again and again from the same disease.
6. **Mental health** - the aim of education in mental health is to help people to keep mentally healthy and to prevent mental breakdown.
 - a. People should enjoy their relationship with others and learn to live and work without mental breakdown.
 - b. Importance of mental health are mother after child birth, child entry first time in school, decision about future carrier.
7. **Prevention of accidents** - accidents are a failure of the complexity of modern life .
 - a. Accidents occur in three main areas ; the home, road and place of work.
 - b. Safety education should be directed to these areas
 - c. Concern department engineering and also the responsibility of the police department to enforce the rules of the road safety
 - d. The predominant factor in accidents is carelessness and the problem can be tackled through health education.
 - e. Management must provide a safe environment and promote general orders and cleanliness.
8. **Use of health services** - many people particularly in rural areas do not know what health services are available in their community.
 - a. Studies shows that the public attitude towards in health services is still apprehensive.
 - b. There is a communication gap between the public and the state health administration in the form of "feedback" for further improvement of health services.

HEALTH EDUCATION AND PROPAGANDA

Health education	Propaganda or publicit
1. Knowledge and skills actively required	1. Knowledge installed in the minds of the people
2. Makes people think for themselves	2. Prevents or discourage thinking by ready-made slogans
3. Disciplines primitive desires	3. Arouses and stimulates primitive
4. Develops reflective behaviour,	

Trains people to use judgement before acting. - Appeals to reason. - Develops individuality, personality and self-expression. - Knowledge acquired through self-reliant activity. - The process is behaviour centred - aims at developing favourable attitudes, habits and skills	desires - Develops reflexive behaviour, aims at impulsive actions - Appeals to emotion - Develops a standard pattern of attitudes and behaviours according to mould used - Knowledge is spoon - fed and passively received - The process is information centred - no change of attitude or behaviour designed
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PRINCIPLE OF HEALTH EDUCATION

- **Credibility** - it is the degree to which the message to be communicated is perceived as trustworthy by the receiver
- **Interest** - it is a psychological principle that people are unlikely to listen to those things which are not to their interest
- **Participation**
- **Motivation** - in every person, there is a fundamental desire to learn. Awakening this desire is called motivation
- **Comprehensive** - in health education, we must know the level of understanding, education and literacy of people to whom the teaching is directed. One barrier to communication is using words which cannot be understood
- **Reinforcement** - few people can learn all that is new in a single period. Repetition at intervals is necessary. If there is no reinforcement, there is every possibility of the individuals going back to the pre- awareness stage.
- **Learning by doing** - learning is an action - process ; not a memorizing one in the narrow sense.
Known to unknown - in health education work, we must process "from the concrete to the abstract" from the particular to the general.
- **Setting an example** - the health educator should set a good example in the things he is teaching.
- **Good human relation** - sharing of information, idea, and feelings happen most easily between people who have a good relationship. Building good relationship with people goes hand in hand with developing communication skills.
- **Leaders** - in the work of health education, we try to penetrate the community through the local leaders - the village headmaster, the school teacher or the political workers. If the leaders are convinced first about a given programme, the rest of the task of implementing the programme will be easy.
- **The attributes of a leader are** - he understands the need and demand of the community, provides proper guidance, takes the initiative. He is receptive to the views and suggestion of the people, self-less and honest, impartial, considerate and sincere.
- **Feedback** - feedback is one of the key concepts of the system approach, the health educator can modify the elements of the system (e.g. message media) in the light of the feedback from audience. For effective communication, feedback is of

paramount importance.

BASIC LISTENING

A process of transforming message/information/advice/instructions, knowledge from one person to another with a view that both have common interpretation and understanding.

OBJECTIVES OF LISTENING

- According to GALVIN (1985), listening serves multiple purposes :
 - Listening for enjoyment - engaging attentively with radio and television programs can enrich our appreciation and enhance our own language skills.
 - Listening for information - this type of listening help us solve problems, follow directions, stay updated with current events, and understand other's viewpoints. It also supports conversations and provides examples that aid in expanding ideas in speeches, letters and other written forms.

FORMAL AND INFORMAL WAYS OF COMMUNICATION

- Formal channels are in-built in any organisation and are an integral part of functioning of it. They are mandated and are documented for future reference
- Informal communication reveals that for a smooth functioning of any organisation this communication inevitable. It helps day to day problem.

SELECTIVE LISTENING (FOCUSING ON SPECIFIC ELEMENTS)

- Selective listening involves concentrating on particular aspects of language one at a time. Listeners should isolate and attend to a single feature or a group of related features before moving on to others.
- This method covers key language components such as phonetics vocabulary and grammar (including morphology and syntax)

INTENSIVE LISTENING (TARGETING DETAILED UNDERSTANDING)

- To develop learner's ability to grasp detailed meanings and recognize specific language features like vocabulary, grammar, or pronunciation, educators should engage them in intensive listening exercise. This approach sharpens attention to fine details within spoken language.

EXTENSIVE LISTENING (GRASPING THE OVERALL MESSAGE)

- Activities like listening to stories, rhymes, songs, TV commercials, poems, fairy tales, and ligands provide rich opportunities for extensive listening. This practise encourages natural development of listening skills by exposing learners to language in varied, meaningful contexts.

OBJECTIVES OF TEACHING LISTENING SKILLS

- Distinguish fundamental English sounds, including vowels, consonants and clusters
- Recognize key patterns in word stress, sentence stress, and intonation
- Comprehend the meaning of words, phrases and sentences within their context.
- Interpret statements, questions, instructions and commands accurately.
- Follow and respond appropriately to both simple and complex oral instructions, whether delivered face-to-face or over the phase.

- Grasp the main ideas and important details in straightforward spoken narratives and descriptive passages
- Accurately follow oral directions
- Understand the core message and overall essence of spoken content.
- Sustain attentive listening for reasonable duration
- Enjoy and appreciate auditory materials such as poems, songs, parodies and music
- Engage with and comprehend discussions on a variety of topics

METHODS OF ENHANCING LISTENING SKILLS

- Engage in exercises focusing on the phonological aspects of English.
- Practise listening comprehension through various activities such as -
 - Dictation drills
 - Audio recording tasks
 - Listening attentively to passages
 - Extracting the main message from spoken content
 - Responding physically to prompts
 - Creating objects based on instructions
 - Marking items off list
 - Completing grids and family trees
- Utilize practical formats like-
 - Filling out biodata sheets, bank or passport forms, railway reservation forms, and interpreting timetables
 - Following flow charts
 - Navigating using maps
- Develop predictive listening skills and practice summarizing spoken information.
- Solve problems through listening comprehension exercises where students hear a problem description and respond by performing the required tasks.

FUNCTION OF COMMUNICATION

Effective communication enhance interpersonal relationships build on trust and teamwork, prevents costly mistakes, motivates and gets things done.

1. To make aware about
2. To develop common understanding
3. To condition mind as to get right perception
4. To motivate and support desired behaviour
5. To motivate and boost up morale
6. To avoid doubts, misunderstanding confusion and biases.

VERBAL COMMUNICATION

- Any communication where language is used.
- Written - any form of writing is used, such as letters, SMS, emails etc. intended for one or more receivers. They have the advantages of having a permanent nature and ability to convey complex and detailed messages. Many mostly formal communication, but can be informal too.
- Oral - spoken word. Intended for one or more receivers. They have the advantages of interactivity, feedback, control over receiver and personal customization. Mostly informal communication.

- Communication can be either one to one (interpersonal communication) or one to many (group communication or mass communication like in emails, meetings)
- Effective interpersonal communication require three important skills-
 - Effective speaking
 - Effective listening
 - Showing appropriate emotion
- Speaking skills has two elements
 1. Verbal component
 - a. Clear - simple
 - b. Concise - short
 - c. Comprehensive - cover all important aspects
 - d. Candid - specific and no ambiguity. It is art itself and requires much practice
 2. Non - verbal communication
 - a. Body language (kinesics)
 - b. Touch (haptics)
 - c. Use of space and time (proxemics and chronemics)
 - d. Tone and voice modulation (paralanguage)

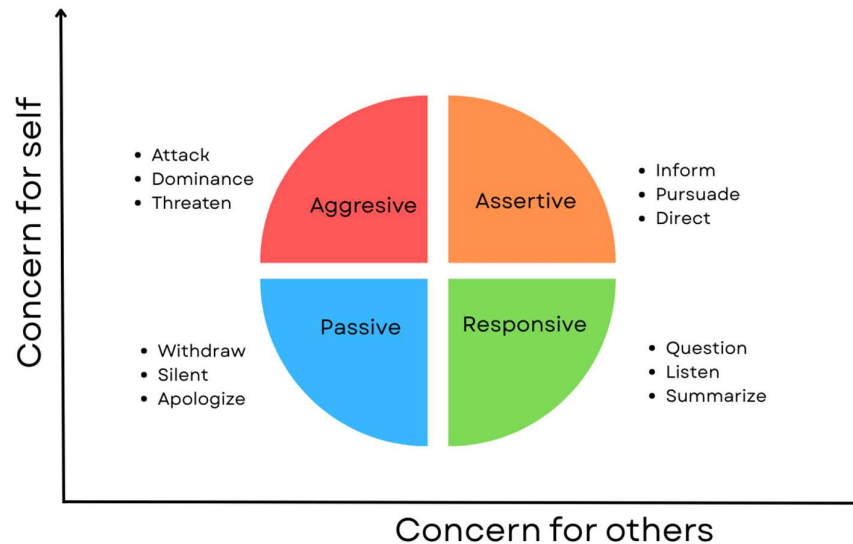
EFFECTIVE LISTENING

- There are some useful guideline to be good listener, a very important communication tool.
- Be patient, avoid trying to guess what someone is about to say.
- Allow the speaker finish their sentence
- Do not interpret or talk over someone else when they are speaking
- Look at the person who is speaking, making eye contact
- Show that you are listening by using nonverbal reactions, such as nodding, smiling, and facial expressions.
- Show curiosity by asking relevant probing questions
- Make a note of any questions you would like to ask after the speaker has finished
- Repeat what has been told said to ensure understanding
- Do not fidget
- Showing appropriate emotions - three important feelings are reflected in the communication -
 - Empathy - when someone places at same level with the individual, understand his/her situation.
 - Sympathy - when one person is paternalistic toward another
 - Apathy - no emotion at all, makes communication ineffective
- Appropriate emotions while speaking helps in communication of the message better.

COMMUNICATION STYLES

- Communication style is a way in which an individual interacts and exchanges information with others
- It is based on the mix of two concerns of a communication
- Concern of self - attitude of communication's concern is for own ideas, thoughts and opinion.

- Concern of others - thought communicator is concerned for other's ideas, thoughts and wants.
- Communication style is divided into four -
 - Assertive - high concern for self and high concern for others
 - Aggressive - high concern for self and low concern for others
 - Responsive - low concern for self and high concern for others
 - Passive - low concern for self and low concern for others



ACTIVE LISTENING

- Active listening means being deeply engaged in and attentive to what the speaker is saying
- It requires far more listening than talking
- Your goal as an active listener is to truly understand the speaker's perspective (regardless of whether you agree) and to communicate that understanding back to the speaker so that he or she can confirm the accuracy of your understanding

PARAPHRASING

- Restate the same information, using different words to more concisely reflect what the speaker said
- Test your understanding of what is heard by communicating your understanding of what the speaker said. Allow the speaker to hear and focus on his or her thoughts. Allow the speaker that you are trying to understand his/her message and perceptions. Encourage the speaker to continue speaking.

RESPONSES

- What I'm hearing is
- Sounds like you are saying
- I am not sure I am with you but
- If I am hearing you correctly
- So as you see it
- It sounds like what's most important to you is

CLARIFYING

- Invite the speaker to explain some aspect of what she or he said.
- Give the speaker opportunity to elaborate and clarify what was said.

RESPONSES

- I am not sure, I quite understand or do you mean that?
- Can you say more about it?

INTERVIEW SKILLS

At least two experts meet with the candidate

Purpose - to assess the candidate's fit for the role

- evaluation focuses on subject expertise, skills and key behavioural traits within a brief timeframe.

TYPES OF INTERVIEW

Panel interview - A group of three or more specialists engage directly with the candidate across table.

Face-to-face interview - a one-to-one meeting between the interviewer and the candidate.

- Typically more casual in tone
- Creates a closer connection between interviewer and the candidate
- Generally fosters a more comfortable and less intimidating atmosphere as compared to the panel interviews.

Pilot interviews -

- Reviews your cv and required skills
- Research the company
- Visit the company website
- Check annual reports
- Read business news and magazines
- Contact employer directly.

carefully, and respond thoughtfully to the interviewer's questions

- Practice honesty : if you're unsure about an answer, it's best to be truthful rather than guessing during the interview.

Know the company

- Core products and services offered
- Yearly revenue, net income and shareholder returns
- Key market rivals
- Geographic region served
- Recently introduced products or services.

Non-verbal or body language

- Maintain an upright posture without leaning on the desk or slumping in your seat, but stay relaxed rather than rigid.
- Establish eye contact when speaking to engage your audience, yet avoid staring continuously to keep it natural
- Limit excessive hand gestures and avoid frequently changing facial expressions that don't match your spoken words.

During interview employer evaluate

- Cultivate self-assurance : approach your interviewer with a strong sense of confidence to make a positive impression
- Practice communication skills : speak clearly in English, listen

EXPECTED QUALITIES OF THE CANDIDATE

- Mastering verbal communication

- Use English free from influence of your mother tongue for clearer understanding.
- Aim for a smooth, natural flow when speaking.
- Articulate each sentence distinctly to ensure your message to understood.
- Adjust your volume - raise your voice to emphasize key points or lower it to capture the audience's attention during important opinions

REJECTED BY THE COMMITTEES

- Responses lack clarity and often stray off-topic.
- Exhibit noticeable restlessness : minimal eye contact and nervous habits like hair twirling or nail biting.
- Displays an absence of genuine emotion or honesty
- Offers shallow comments aimed solely at impressing the interviewer, especially regarding project experience.

DO's

- Plan a trial visit to the interview venue beforehand to familiarize yourself with the location.
- When asked to complete a job application, ensure your responses are neat, through and precise.
- Carry multiple copies of your resume to the interview for convenience.
- Offer a warm greeting to your interviewer and deliver a confident, firm handshake.
- Wait patiently to be invited to sit down rather than taking a seat immediately.
- Maintain strong and confident eye contact with your interviewers.

- Express genuine excitement about the role and the organisation.
- Clearly and honestly communicate your strengths and positive attributes.
- Highlight your accomplishments to leave a lasting impression.

DON'Ts

- Steer clear of sensitive or divisive subjects.
- Always provide honest, straightforward and concise answers, never fabricate information.
- Refrain from speaking ill of past colleagues, managers or employee.
- Avoid responding with just "yes" or "no". whenever possible, elaborate by sharing stories or examples that highlight your abilities, skills and perseverance.
- Avoid discussing salary, vacation time, bonuses, retirement plans, or any other benefits until you have officially received a job offer.
- Refrain from mentioning personal matters or family related issues during interview
- Do not repeat your employment history verbally, as it is already detailed in your resume.

AFTER INTERVIEW

- Express your gratitude for being invited to the interview through a formal letter.
- Convey your eagerness to receive further communication from them.

INTERVIEW QUESTION

- Can you share a bit about your background and experience?
- What prompted you to move on from your last position?

- c) What type of role are you currently seeking?
- d) What aspects of this position attract you the most?
- e) Why do you believe you would excel in this role?
- f) What would you say are your key strengths?
- g) What areas do you feel you could improve upon?
- h) If offered this job, would you be ready to accept it?
- i) How familiar are you with our company and its mission?
- j) If you joined us, how long do you envision staying?
- k) Do you have any concerns about your age, experience, or background in relation to this role?

TELL ME ABOUT YOURSELF

- The question you're most likely to encounter in interviews.
- Share your educational background.
- Highlight your key skills
- Outline your qualifications
- Discuss relevant experiences and previous roles connected to the job you're applying for.
- Aim to keep your response within 3 minutes.
- Craft a written response in advance and practise delivering it confidently.

WHAT DO YOU THINK YOU ARE GOOD AT THIS JOB?

- I want a job I enjoy, with good people, where I can contribute and face challenges that let me use my skills.

WHAT ARE YOUR STRENGTH

- Depth of experience in your field.
- Skill in setting priorities effectively.

- Strong analytical and problem - solving abilities.
- Capacity to perform well under stress.
- Consistent focus and dedication to projects
- Demonstrated professional competence
- Proven leadership capabilities.
- Maintaining a constructive and optimistic mindset

WHAT ARE THE WEAKNESS

- Frame your weakness as potential strength.
- Emphasize your dedication to completing assigned tasks, a commitment you also expect from your colleagues.
- While this approach may sometimes impact interpersonal dynamics, it ultimately ensures high-quality results.
- This mindset aligns well with the responsibilities and expectation of a team leader.